NCLEX NGN CASE STUDY STUDIO · QUESTION BANK REPORT

NGN Case Study Inventory

Case studies only (stand-alone questions excluded) · 14 October 2026, after today's fixes · NURS 1017 and NURS 1021

52	32	20	51	22
case studies in the bank	in NURS 1017	in NURS 1021	ready for students	changed by Claude: need your review

Changed today

University of Maryland CS3 to CS21: the open review notes were resolved as far as possible, and all Maryland case studies are now available. "NURS 1017 Unit 11 Case Study 1" (DKA, was "Case Study 2") and "NURS 1021 Unit 2 Case Study 1" (heart failure, was "Case Study 1") have new titles. The two duplicates were deleted. The Unit 7 stroke bow-tie is now a stand-alone question. Answer options and drop-down choices that listed a correct answer first are now in a random order. Everything Claude changed carries a "Review" note in the studio.

Your work list

22	1	4
1. Changed by Claude: double-check	2. Unfinished (set aside)	3. Other reviews you planned

1. Changed by Claude: please double-check 22

Students can use these now. In the studio they show a "Review" tag in the Question bank (Status filter: "Needs my review") and a "Needs your review" chip in the editor with the same list. When you are satisfied, choose "Mark as reviewed" in the editor and save. "Please confirm" marks a decision taken from the source that you may want to change.

CASE STUDY	COURSE AND UNIT	WHAT WAS CHANGED: PLEASE DOUBLE-CHECK
University of Maryland - CS4 Breast cancer: pancytopenia after chemotherapy, arm lymphedema	NURS 1017 Unit 4 Neoplasia	• Screen 2: the author's key kept (numbness/tingling → lymphedema and DVT; HR 104 → cellulitis only). Please confirm. • Screen 2: the weak row "Sodium 148 mmol/L" is now "Axillary lymph node removal 45 days ago" (lymphedema). • Screen 4: amoxicillin is now NOT anticipated (no signs of infection; neutropenic client); it was removed from the 1130 orders note and the discharge prescriptions. • Discharge acetaminophen changed from twice daily to every 6 hours as needed. • Screen 6 "ice bag = no understanding" kept (avoid temperature extremes on the affected arm). Please confirm.
University of Maryland - CS5 Burns: burn shock during fluid resuscitation	NURS 1017 Unit 5 Integumentary Disorders and Burns	• Burns now encircle both legs (front and back), so 36% TBSA (and the 14,400 mL Parkland total) matches the Rule of Nines. • Intake and output: the 2300 IV rate is now 1584 mL/hr (+20% for the 10 mL/hr urine output at 2200, per the titration order). • Albumin order kept as "systolic BP < 80 or diastolic BP < 50". Please confirm "or" vs "and".
University of Maryland - CS18 Spine surgery: L2–L4 laminectomy complications	NURS 1017 Unit 6 Musculoskeletal Disorders	• Screen 5: "check the dressing in the mirror" is now "have a family member check the dressing daily". • Senna (a stimulant laxative) replaced by docusate (a stool softener), so the teaching about a stool softener is accurate. • The CS18 bow-tie (stand-alone, still hidden) still places a wound infection on day 1–2; not changed.
University of Maryland - CS7 Compartment syndrome after ORIF of a tibia-fibula fracture	NURS 1017 Unit 6 Musculoskeletal Disorders	• Screen 5: the vital-signs task now matches the STAT order (every 15 minutes) and is not delegated (unstable client). • Screen 6: rationale now flags the cold, pulseless limb as urgent. • RR 29 at 0830 changed to 20 (likely typo). • Screen 6 colour keyed Improved (flow sheet N at 1245); the CS7 trend (stand-alone) shows pale (P) for the same time. Please confirm.
University of Maryland - CS19 Stroke in a 26-year-old on a combined hormonal contraceptive	NURS 1017 Unit 7 Neurological Disorders	• Heart rate changed from 99 to 112 beats/min, so it is truly abnormal (it is keyed on screen 1). • Unexplained labs normalized: Hct 0.40 L/L, platelets 250 × 10^9/L, PT 11 seconds. • Discharge apixaban (no atrial fibrillation) replaced by antiplatelet therapy (aspirin). • Screen 6: the physical-therapist statement is now "I will not need any more therapy" (no understanding). • The CS19 bow-tie (stand-alone, still hidden) keys oxygen with no SpO_2 charted; not changed.
NURS 1017 Unit 11 Case Study 1 Diabetic ketoacidosis in a 20-year-old woman	NURS 1017 Unit 11 Endocrine Disorders	• Title made up: "NURS 1017 Unit 11 Case Study 1" (was "Case Study 2"), with a new description. Please check both. • Screen 2 row typo "Polyruria" corrected.
University of Maryland - CS15 Type 2 diabetes with DKA on an SGLT2 inhibitor	NURS 1017 Unit 11 Endocrine Disorders	• Screen 4: insulin glargine and the indwelling catheter are now "Nonessential" (were contraindicated). • Pulse oximetry is now normal on room air and no oxygen is started (the 89% was unexplained with clear lungs). • Sitagliptin/metformin changed to twice daily; screen 1 rationale mentions empagliflozin as a DKA risk. • The corrected blood gas ($PaCO_2$ 25, HCO_3^- 10), the KCl concentration (20 mmol/L) and the changed key "pursed-lip breathing = contraindicated" from the conversion still need your confirmation.
University of Maryland - CS21 Thyroid storm in a 32-year-old	NURS 1017 Unit 11 Endocrine Disorders	• Room-air SpO_2 changed from 97% to 92% (0900, 0910) so oxygen is indicated; screen 6 now keys oxygenation as Improved. • TSH changed from 0.27 to < 0.01 mU/L (suppressed, as expected in thyroid storm). • Propranolol 60 mg PO every 4 hours added to the orders and as a correct option on screen 4. • Screen 5 key (oxygen and the ECG first) still comes from the source rationale (its highlighting was unreadable). Please confirm. • Methimazole kept (propylthiouracil is often preferred in thyroid storm).

CASE STUDY	COURSE AND UNIT	WHAT WAS CHANGED: PLEASE DOUBLE-CHECK
University of Maryland - CS10 Deep vein thrombosis: heparin protocol (DVT II)	NURS 1021 Unit 1 Blood Disorders	• Pedal pulse is now 2+ (a weak pulse is not a DVT sign); screen 1 is now "select 3"; the screen 2 row "Decreased pedal pulse" is now "Fever" (infection). • Bedrest only until heparin starts; screen 4 keys walking as tolerated once anticoagulated as indicated (CHEST/ASH). • Compression stockings removed from the orders; screen 4 row is now a sequential compression device on the DVT leg (contraindicated); screen 5 row is now "Weigh the client" (immediately). • Weight 80 kg added for the weight-based heparin. • Screen 2 row labels were rebuilt from the source rationale (the source table was blank). Please confirm.
NURS 1021 Unit 2 Case Study 1 Acute heart failure: 72-year-old man in respiratory distress (ED)	NURS 1021 Unit 2 Cardiovascular Disorders	• Title "NURS 1021 Unit 2 Case Study 1" and description made up. Please check both. • Screen 3: the first drop-down had three empty choices; now high-Fowler's (correct, per the rationale), supine and Trendelenburg. • Screen 6 rationale no longer numbers its points 1–3 (they looked like option numbers).
University of Maryland - CS13 Heart failure with hypokalemia (clinic)	NURS 1021 Unit 2 Cardiovascular Disorders	• Screen 6 row "Thirst" (no day-1 baseline) is now "Worry about his health" (unchanged). • Hemoglobin changed from 135 to 115 g/L to fit the hematocrit of 0.346. • Hydrochlorothiazide 25 mg twice daily replaced by furosemide 40 mg twice daily (loop diuretic).
University of Maryland - CS6 Chest pain: anterior STEMI, ECG and biomarkers	NURS 1021 Unit 2 Cardiovascular Disorders	• Room-air SpO_2 changed from 93% to 88%, so starting oxygen fits current ACS guidelines (oxygen when SpO_2 < 90%). • Enoxaparin written as the fibrinolysis regimen (30 mg IV bolus, then 1 mg/kg SC, maximum 100 mg); drop-down choice updated. • Added that the hospital has no catheterization lab, which explains fibrinolysis within 30 minutes. • Screen 3 key "blockage" kept (the source had no key); rationale now explains plaque rupture vs blockage. Please confirm. • The CS6 bow-tie (stand-alone, still hidden) still keys oxygen and aspirin wording; not changed.
University of Maryland - CS2 Tuberculosis: contact tracing, diagnosis, infection control	NURS 1021 Unit 3 Respiratory Disorders	• Isoniazid is shown as 300 mg PO daily (the source said 1000 mg, above the maximum). Confirm the regimen and dose.
University of Maryland - CS20 Tension pneumothorax after rib fractures	NURS 1021 Unit 3 Respiratory Disorders	• Chest X-ray moved to after decompression and chest tube; screen 4 option now "Obtain a chest X-ray after the chest tube is placed". • Screen 4: the urinary catheter option is now "Prepare the client for a thoracentesis" (not anticipated). • Muffled heart sounds (a tamponade sign) replaced by jugular venous distention in the chart and the screen 2 grid. • Screen 5: drainage is measured hourly at first (was every shift); unexplained right-sided crackles removed; 1600 SpO_2 now 97% on 2 L/min. • Screen 3 uses rationale scoring as in the source. The CS20 bow-tie (stand-alone, still hidden) still keys the chest X-ray as an action; not changed.
University of Maryland - CS3 Acute respiratory failure with pneumonia (ICU)	NURS 1021 Unit 3 Respiratory Disorders	• IV amoxicillin (no IV form) replaced by piperacillin-tazobactam 4.5 g IVPB every 6 hours. • Blood culture now "gram-negative rods" (was cocci). • SaO_2 changed from 96% to 94% to fit the PaO_2 of 75 mm Hg. • Lactate changed from 4.5 to 2.4 mmol/L (a lactate of 4.5 would have been one of the most urgent findings on screen 1); rationale sentence added.
University of Maryland - CS8 COPD exacerbation (COPD I)	NURS 1021 Unit 3 Respiratory Disorders	• Digital clubbing removed from the chart (not a feature of COPD); screen 1 option is now "Barrel chest appearance" and the screen 2 row is now "Green-tinged sputum" (COPD and pneumonia). • Low-flow oxygen (1 L/min, target 88–92%) now starts at 1015; the 1045 reading is 88% on 2 L/min. • Screen 4: incentive spirometer is now NOT included during the acute exacerbation. • Acetaminophen order changed to every 6 hours as needed for mild pain or fever (the client had no back pain).

CASE STUDY	COURSE AND UNIT	WHAT WAS CHANGED: PLEASE DOUBLE-CHECK
University of Maryland - CS9 COPD exacerbation with pneumonia (COPD II)	NURS 1021 Unit 3 Respiratory Disorders	• Screen 2: clubbing is now keyed "Not associated with COPD", with an explanation. • Inhaler written as fluticasone/salmeterol 250/50 mcg dry powder inhaler. • Discharge: levofloxacin 750 mg daily for 5 days; roflumilast started at 250 mcg for 4 weeks.
University of Maryland - CS14 HIV with Pneumocystis pneumonia after stopping ART	NURS 1021 Unit 4 Inflammation and Immune Disorders	• Screen 4: ibuprofen replaced by acetaminophen (raised creatinine); rationale mentions adjunct corticosteroids for hypoxemic PCP. • Sodium changed from 148 to 144 mmol/L (it was not keyed as concerning). • Screen 6 "I can stop wearing a mask when I feel better = no understanding" kept. Please confirm.
University of Maryland - CS12 Gastroesophageal reflux disease (ED)	NURS 1021 Unit 6 Gastrointestinal Disorders	• An ECG and troponin are now ordered and documented as normal, so chest pain is ruled out before GERD is diagnosed. • Morphine replaced by pantoprazole 40 mg IV; screen 4 option "Administer analgesics" is now "Administer a proton pump inhibitor". • Potassium replacement ordered and started for K 3.0 mmol/L.
University of Maryland - CS16 Liver failure: cirrhosis, ascites, hepatic encephalopathy	NURS 1021 Unit 6 Gastrointestinal Disorders	• Screen 4: the acetaminophen row is now ibuprofen (contraindicated in cirrhosis); acetaminophen ≤ 2 g/day is preferred. • Screen 4: point-of-care glucose is now Indicated (malnourished, glucose 3.9 mmol/L). • Metoprolol XL replaced by propranolol 20 mg twice daily (nonselective beta-blocker for portal hypertension). • No creatinine is charted; consider adding one.
University of Maryland - CS11 End-stage renal disease: peritoneal dialysis and peritonitis	NURS 1021 Unit 7 Urinary Disorders	• Screen 4 row is now "Vancomycin IV as the only antibiotic" (not appropriate); rationale explains empiric gram-positive and gram-negative coverage. • Enoxaparin rationale now says unfractionated heparin is preferred in ESRD. • Delegation uses "practical nurse (LPN/RPN)"; keys follow the source. Please confirm against Ontario scope. • Dialysate reference ranges added (WBC < 100 cells/µL, neutrophils < 50%). • Screen 6 "exit site open to air = does not understand" kept; practice varies once the site is healed. Please confirm with your program.
University of Maryland - CS17 Prostate cancer: hemorrhagic shock after prostatectomy	NURS 1021 Unit 9 Reproductive Disorders	• Ketorolac (an NSAID, in a bleeding client) replaced by morphine 2 mg IV. • Continuous bladder irrigation and catheter traction removed (they belong to TURP, not robotic radical prostatectomy); the screen 5 highlight passage no longer lists them. • Screen 3 key "cardiovascular assessment" kept (the source had no key); rationale strengthened. Please confirm.

2. Unfinished 1

Not visible to students until the missing content is added.

CASE STUDY	COURSE AND UNIT	WHAT IS MISSING
Lower Extremity Fracture In progress (set aside at your request)	NURS 1017 Unit 6 Musculoskeletal Disorders	• Chart tabs are empty • Screens 3, 4 and 5 have no question text; screen 4 has no answer key • Screen 2: a blank matrix row; screen 6: a drop-down without exactly one correct answer • No rationales on screens 2 to 6

3. Other reviews you planned 4

Ready case studies with a review you asked for earlier (no "Review" tag in the studio).

CASE STUDY	COURSE AND UNIT	TO DO
NURS 1017 Unit 5 Case Study 6 Flame burns with suspected inhalation injury (OpenStax)	NURS 1017 Unit 5 Integumentary Disorders and Burns	• Review the new rationales and figures (October 2026)
NURS 1017 Unit 5 Case Study 7 Herpes zoster ophthalmicus, immunosuppressed older adult (OpenStax)	NURS 1017 Unit 5 Integumentary Disorders and Burns	• Review the new rationales and figures (October 2026)
NURS 1017 Unit 5 Case Study 8 Psoriasis progressing to erythroderma; infliximab (OpenStax)	NURS 1017 Unit 5 Integumentary Disorders and Burns	• Review the new rationales and figures (October 2026)
NURS 1017 Unit 3 Case Study 4 Duchenne muscular dystrophy in a 4-year-old boy	NURS 1017 Unit 3 Genetic and Developmental Disorders	• Clinician review of the medical content (flagged when the case was written)

Counts by course and unit

UNIT	CASE STUDIES	READY	NEED YOUR REVIEW	NOT READY
NURS 1017				
Unit 1 Introduction to Pathophysiology	3	3	–	–
Unit 2 Cellular Basis of Disease	3	3	–	–
Unit 3 Genetic and Developmental Disorders	4	4	–	–
Unit 4 Neoplasia	6	6	1	–
Unit 5 Integumentary Disorders and Burns	9	9	1	–
Unit 6 Musculoskeletal Disorders	3	2	2	1
Unit 7 Neurological Disorders	1	1	1	–
Unit 8 Pain	–	–	–	–
Unit 9 Disorders of the Eyes and Ears	–	–	–	–
Unit 10 Stress and Disease	–	–	–	–
Unit 11 Endocrine Disorders	3	3	3	–
NURS 1017 total	**32**	**31**	**8**	**1**
NURS 1021				
Unit 1 Blood Disorders	2	2	1	–
Unit 2 Cardiovascular Disorders	3	3	3	–
Unit 3 Respiratory Disorders	7	7	5	–
Unit 4 Inflammation and Immune Disorders	1	1	1	–
Unit 5 Leukemias and Lymphomas	–	–	–	–
All case studies	**52**	**51**	**22**	**1**

UNIT	CASE STUDIES	READY	NEED YOUR REVIEW	NOT READY
Unit 6 Gastrointestinal Disorders	5	5	2	–
Unit 7 Urinary Disorders	1	1	1	–
Unit 8 Fluid, Electrolytes, and Acid-Base Imbalances	–	–	–	–
Unit 9 Reproductive Disorders	1	1	1	–
NURS 1021 total	**20**	**20**	**14**	**0**
All case studies	**52**	**51**	**22**	**1**

No case studies yet: NURS 1017 Units 8 (Pain), 9 (Disorders of the Eyes and Ears) and 10 (Stress and Disease); NURS 1021 Units 5 (Leukemias and Lymphomas) and 8 (Fluid, Electrolytes, and Acid-Base Imbalances).

All case studies

NURS 1017

CASE STUDY	SCREENS	STATUS
Unit 1: Introduction to Pathophysiology 3		
NURS 1017 Unit 1 Case Study 1 Foodborne gastroenteritis in a 19-year-old student (campus clinic)	6	Ready
NURS 1017 Unit 1 Case Study 2 Chronic work stress in a 44-year-old vice-principal (stress response)	6	Ready
NURS 1017 Unit 1 Case Study 3 Early rheumatoid arthritis in a 29-year-old woman (inflammation, autoimmunity)	6	Ready
Unit 2: Cellular Basis of Disease 3		
NURS 1017 Unit 2 Case Study 1 Cellular adaptation: Barrett's metaplasia, bronchial metaplasia, LV hypertrophy	6	Ready
NURS 1017 Unit 2 Case Study 2 Cell injury: alcohol-related hepatic steatosis and hydropic swelling	6	Ready
NURS 1017 Unit 2 Case Study 3 Cell death: dry gangrene progressing to gas gangrene and septic shock	6	Ready
Unit 3: Genetic and Developmental Disorders 4		
NURS 1017 Unit 3 Case Study 1 Chromosomal disorders: newborn with Down syndrome (vs Turner, Klinefelter)	6	Ready
NURS 1017 Unit 3 Case Study 2 Preconception counselling: cystic fibrosis, Marfan syndrome, hemophilia	6	Ready
NURS 1017 Unit 3 Case Study 3 Teratogens: newborn with fetal alcohol spectrum disorder	6	Ready
NURS 1017 Unit 3 Case Study 4 Duchenne muscular dystrophy in a 4-year-old boy	6	Ready
Unit 4: Neoplasia 6		
NURS 1017 Unit 4 Case Study 1 Benign vs malignant tumours: leiomyoma and invasive ductal carcinoma	6	Ready

CASE STUDY	SCREENS	STATUS
NURS 1017 Unit 4 Case Study 2 Colorectal adenocarcinoma: screening and carcinogenesis	6	Ready
NURS 1017 Unit 4 Case Study 3 Non-small cell lung cancer: biopsy, TNM staging, PET	6	Ready
NURS 1017 Unit 4 Case Study 4 Pancreatic cancer: cachexia, mucositis, febrile neutropenia	6	Ready
NURS 1017 Unit 4 Case Study 5 Small cell lung cancer with paraneoplastic SIADH	6	Ready
University of Maryland - CS4 Breast cancer: pancytopenia after chemotherapy, arm lymphedema	6	Ready Review
Unit 5: Integumentary Disorders and Burns 9		
NURS 1017 Unit 5 Case Study 1 Photoaging and skin lesions: basal cell carcinoma and melanoma	6	Ready
NURS 1017 Unit 5 Case Study 2 Impetigo (child) and herpes zoster (sibling)	6	Ready
NURS 1017 Unit 5 Case Study 3 Systemic lupus erythematosus vs other rashes	6	Ready
NURS 1017 Unit 5 Case Study 4 Major burn with inhalation injury; Rule of Nines, Parkland	6	Ready
NURS 1017 Unit 5 Case Study 5 Circumferential burns: escharotomy and grafting	6	Ready
NURS 1017 Unit 5 Case Study 6 Flame burns with suspected inhalation injury (OpenStax)	6	Ready
NURS 1017 Unit 5 Case Study 7 Herpes zoster ophthalmicus, immunosuppressed older adult (OpenStax)	6	Ready
NURS 1017 Unit 5 Case Study 8 Psoriasis progressing to erythroderma; infliximab (OpenStax)	6	Ready
University of Maryland - CS5 Burns: burn shock during fluid resuscitation	6	Ready Review
Unit 6: Musculoskeletal Disorders 3		

CASE STUDY	SCREENS	STATUS
Lower Extremity Fracture Lower extremity fracture (in progress)	6	Not ready
University of Maryland - CS18 Spine surgery: L2–L4 laminectomy complications	6	Ready Review
University of Maryland - CS7 Compartment syndrome after ORIF of a tibia-fibula fracture	6	Ready Review
Unit 7: Neurological Disorders 1		
University of Maryland - CS19 Stroke in a 26-year-old on a combined hormonal contraceptive	6	Ready Review
Unit 11: Endocrine Disorders 3		
NURS 1017 Unit 11 Case Study 1 Diabetic ketoacidosis in a 20-year-old woman	6	Ready Review
University of Maryland - CS15 Type 2 diabetes with DKA on an SGLT2 inhibitor	6	Ready Review
University of Maryland - CS21 Thyroid storm in a 32-year-old	6	Ready Review

NURS 1021

CASE STUDY	SCREENS	STATUS
Unit 1: Blood Disorders 2		
NURS 1021 Unit 1 Case Study 1 Deep vein thrombosis progressing to pulmonary embolism	6	Ready
University of Maryland - CS10 Deep vein thrombosis: heparin protocol (DVT II)	6	Ready Review
Unit 2: Cardiovascular Disorders 3		
NURS 1021 Unit 2 Case Study 1 Acute heart failure: 72-year-old man in respiratory distress (ED)	6	Ready Review
University of Maryland - CS13 Heart failure with hypokalemia (clinic)	6	Ready Review

CASE STUDY	SCREENS	STATUS
University of Maryland - CS6 Chest pain: anterior STEMI, ECG and biomarkers	6	Ready Review
Unit 3: Respiratory Disorders 7		
NURS 1021 Unit 3 Case Study 1 Pneumonia in a 78-year-old woman with atrial fibrillation (NCSBN)	6	Ready
University of Maryland - CS1 Asthma exacerbation in a 23-year-old man (University of Maryland CS1)	6	Ready
University of Maryland - CS2 Tuberculosis: contact tracing, diagnosis, infection control	6	Ready Review
University of Maryland - CS20 Tension pneumothorax after rib fractures	6	Ready Review
University of Maryland - CS3 Acute respiratory failure with pneumonia (ICU)	6	Ready Review
University of Maryland - CS8 COPD exacerbation (COPD I)	6	Ready Review
University of Maryland - CS9 COPD exacerbation with pneumonia (COPD II)	6	Ready Review
Unit 4: Inflammation and Immune Disorders 1		
University of Maryland - CS14 HIV with Pneumocystis pneumonia after stopping ART	6	Ready Review
Unit 6: Gastrointestinal Disorders 5		
NURS 1021 Unit 6 Case Study 1 Ruptured appendix in a 41-year-old man (NCSBN)	6	Ready
NURS 1021 Unit 6 Case Study 2 Splenic laceration and hemothorax after blunt trauma (NCSBN)	6	Ready
NURS 1021 Unit 6 Case Study 3 H. pylori peptic ulcer disease progressing to GI bleed	6	Ready
University of Maryland - CS12 Gastroesophageal reflux disease (ED)	6	Ready Review
University of Maryland - CS16 Liver failure: cirrhosis, ascites, hepatic encephalopathy	6	Ready Review

CASE STUDY	SCREENS	STATUS
Unit 7: Urinary Disorders 1		
University of Maryland - CS11 End-stage renal disease: peritoneal dialysis and peritonitis	6	Ready Review
Unit 9: Reproductive Disorders 1		
University of Maryland - CS17 Prostate cancer: hemorrhagic shock after prostatectomy	6	Ready Review

Ready

Passes the studio's readiness check; shown in the student portal once it opens, and through shared links now.

Review

Content changed by Claude that you have not checked yet. Only you see the note, in the studio.

Answer order

Options and drop-down choices are in a random order, so a correct answer is sometimes first, as on the exam.